

THE

PATIENT: THE SOMATIC GHOST

CYCLE: 28

SOMATIC GHOST



A counter-archive
of the 28th cycle
by Adri M.

Aim for the tails.

THE SOMATIC GHOST

A Counter-Archive of the 28th Cycle

This zine maps the fourteen-day rhapsody of chemotherapy: from liquid fire to first light. It is a tool for navigating poisonous knowledge, a refusal of patience, and a record of the 1% response.

***To my ever-loving family,
Natacha, Zoe, Lorena,
Mauricito, Tito and Manolo.
Without whom I wouldn't be here
to tell this story.***

Adri M.

GRSJ 511 | Winter 2026
<https://www.polivoxia.ca>

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[https://www.polivoxia.ca/
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Colophon (v1.0.2)

Set in Courier New. Compiled with Typst.

Post-processed with Python (Pillow, NumPy).

Pulp effect: ink bleed, directional smudge, grain, misregistration, specks, skew.

Imposed with Plica for 2-up booklet printing.

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*Cúrate, mijita, el dolor
con el sonido del río y la espuma.*

Table of Contents

Intent	1
Manifiesto: Against Patience	3
Activity 1: The Clinic Maze	5
Day 1: The Infusion	6
Day 2: The Pebbles	7
Day 3: The Persistence	8
Activity 2: Letter Soup	9
Day 4: Grounding Ritual	10
Day 5: The Sunday Crash	11
Day 6: The Monday Purge	12
Activity 3: The Somatic Map	13
Day 7: First Light	14
Days 8-14: The Awesome Window	15
Coda	16
The Somatic Glossary	17
References	19

**"How do you tell a story that cannot be told,
yet must be told?"**

• M. NourbeSe Philip

This zine reflects one person's experience. Every
body, every cycle, every journey is different and
equally valid. Nothing here is medical advice.

*Cúrate, mijita, con nuestra luz del sol
y los rayos de la luna.*

*Endulza la mañana con aroma de lavanda
y que venga la calma.*

Intent: The Somatic Counter-Archive

The Problem of the "Patient"

In the clinical archive, to be a **Patient** is to be a site of institutional management. The medical gaze lies rooted in the Gaussian logic of the mean and categorizes bodies based on their proximity to the average. This process often results in the erasure of the individual's agency, transforming the person into a passive vessel for **difficult knowledge**. This institutional erasure mirrors the **silencing** described by Michel-Rolph Trouillot (Trouillot, 2015), where power determines which somatic facts are recorded and which are relegated to the margins of the archive. *The Somatic Ghost* is a refusal of this passivity. It represents a transition from being a **Patient** (one who suffers and waits) to being an **Author** (one who acts and documents).

Why a Zine?

The choice of the zine as a medium is a deliberate decolonial and transdisciplinary intervention. Zines serve as counter-archives for marginalized communities to record living histories that institutions often deny or erase (Eichhorn, 2013; Licona, 2012). By utilizing a hybrid analog-digital aesthetic, this zine mirrors the somatic experience: a mixture of clinical precision and raw, embodied traces. As M. NourbeSe Philip (Philip, 2022) asks regarding silenced histories, this zine explores "how do you tell a story that cannot be told, yet must be told?"

*Cúrate, mijita, el dolor con el calor del sol
y el frío de la luna.*

Mira el mundo con el ojo aquel
que lleva uste' en la frente.

Mapping the Somatic Archive

The subject of this zine is the 14-day rhapsody of a **FOLFOXIRI** chemotherapy cycle. This is not merely a record of side effects; it is a map of resistance. I utilize Manduhai Buyandelger's concept of **poisonous knowledge** (Buyandelger, 2019), understood as knowledge that acts as a destructive force, unsettling the individual's world, transforming it into a methodology of resilience.

Aiming for the Tails

The recurring motif, **Aim for the tails**, is a rejection of the clinical average. In a terminal prognosis, survival is found in the statistical outliers: the 1% response. This zine documents the labor required to exist in those tails: the aggressive diet, the sodium "Purge," and the grounding rituals.

The Somatic Ghost is an act of "re-membering". It is a **descent into the ordinary**, as theorized by Veena Das, where the extraordinary labor of survival becomes the pulse of daily life (Das, 2006). This counter-archive insists that the body is not just a site of catastrophe, but a site of authority, joy, and collective care.

On the critique of the medical system.

This zine critiques the normative gaze of the institution and the reduction of a person to a data point forcibly in the middle of a Gaussian Curve. It challenges the system that sees bodies as statistical objects to be managed rather than as nuanced, contradictory, irreducible human beings. And yet, it is written by someone whose life was saved by that very system. The hospital, the therapy, the protocols, they are the reason this archive exists at all. To critique is not to reject. It is to insist that gratitude and resistance can coexist. It is a call to make things better.

Note: The text in the headers and footers is drawn from the lyrics of "María la Curandera" by Natalia Lafourcade (2022).



Con el vaivén del mar que va y viene, deja que te ame.

Manifiesto: Against Patience

$$f(x) = \frac{1}{\sigma\sqrt{2\pi}} e^{-\frac{1}{2}\left(\frac{x-\mu}{\sigma}\right)^2}$$

The Latin root *patiens* defines the patient as *one who suffers* or *one who waits* (Oxford University Press, n.d.). In the medical archive, to be a **Patient** is to accept a state of passivity, becoming a silent vessel for clinical data and a passive audience to one's own prognosis. I refuse to wait. I refuse to be defined by the act of enduring.

Difficult knowledge is an embodied experience that poses emotional and epistemological challenges (Zembylas, 2014; Lehrer, 2011). To engage with it is to perform the labour of challenging institutions that would rather we remain silent. My attitude is one of authority and action. Even if it hurts, I don't let that suffering be my guide. I am a researcher of my own fragmented flesh, a witness to the descent of the extraordinary into the ordinary.

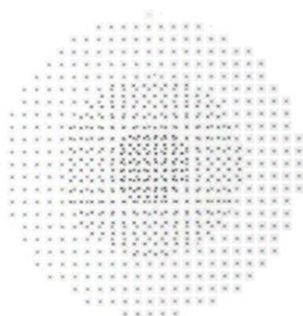
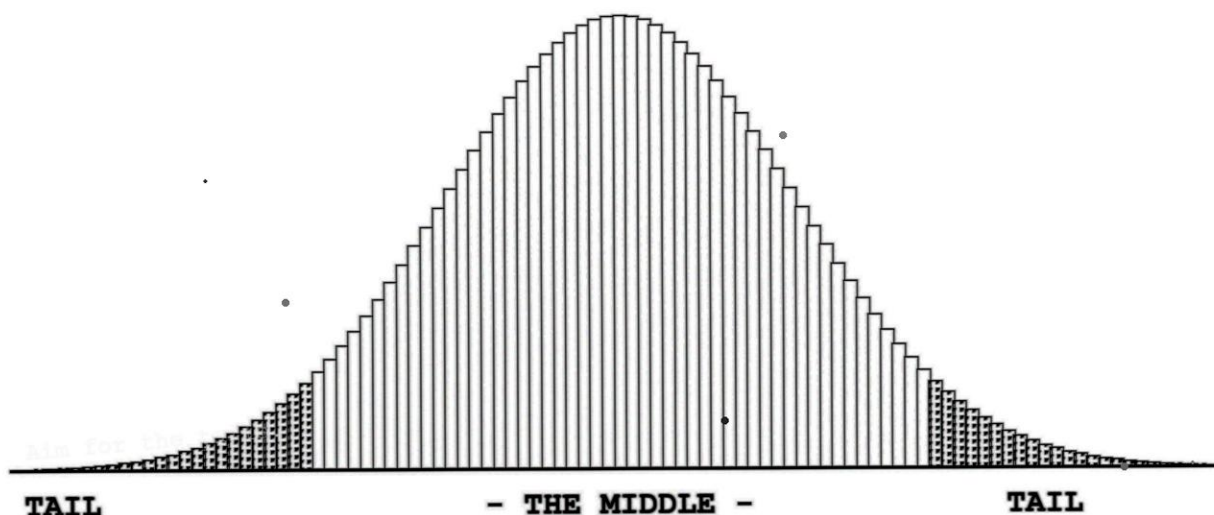
The **Gaussian Distribution** depicted above represents the *normal* curve. In this architecture of probability, the center is where bodies are managed and categorized. The *normal* curve. The *normal* distribution. I do not exist at the mean. I aim for the 1% response that survives a 13cm metastasis. I aim to be the outlier that insists on a future grounded in joy.

*Ponle amor al té, en lugar de azúcar,
toma y mira las estrellas.*

Y recuerda siempre que tú eres la medicina.

Authority is not isolation. My reclamation of this archive is fueled by the collective care of my family and community, who also refuse to let my history be erased by clinical silence. **Aim for the tails.** Stay away from the middle of the curve.

Normal curve / Gaussian distribution

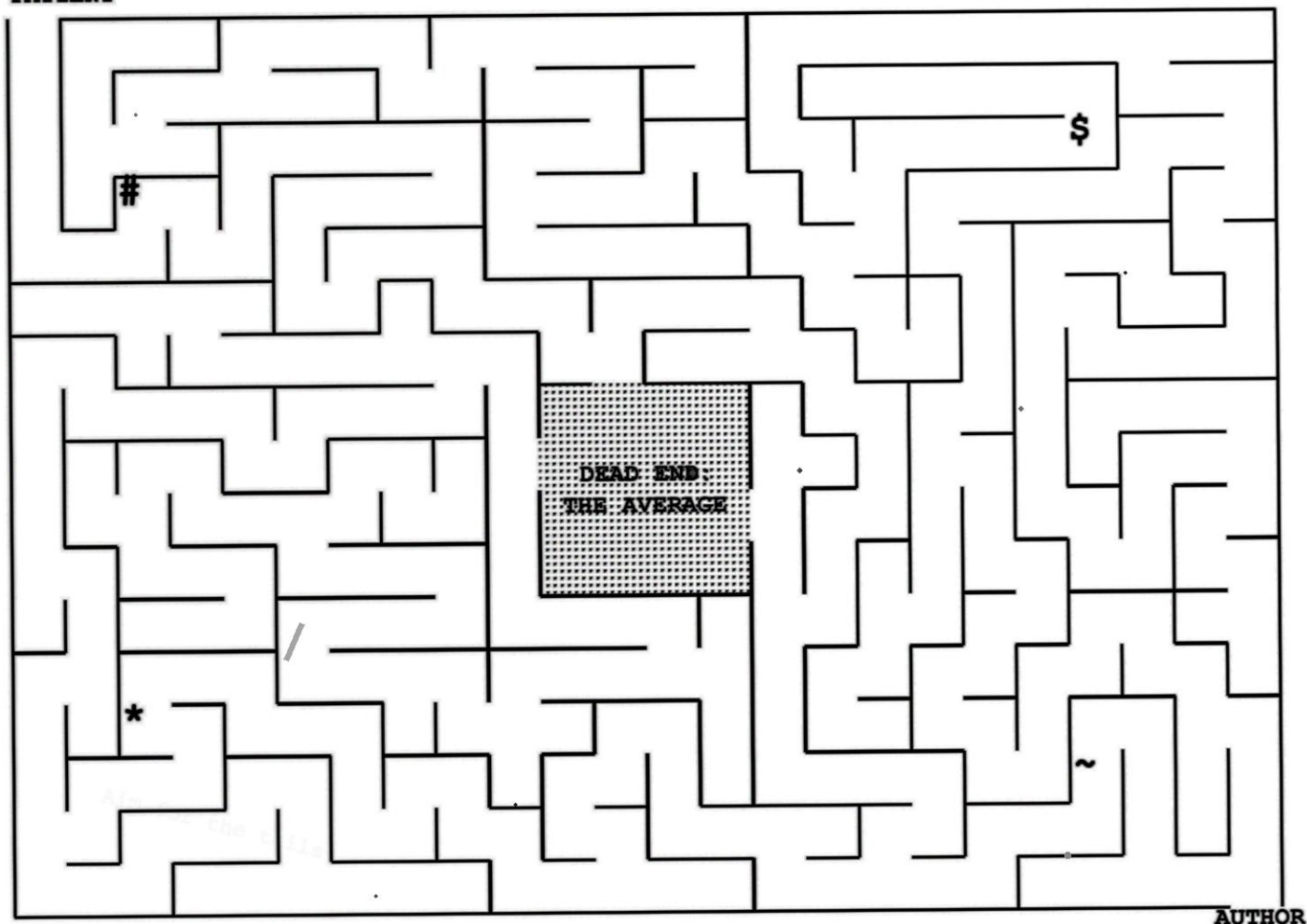


*Cúrate, mi niña, con las hojas de la menta
y la hierbabuena.*

Activity 1: The Clinic Maze

The clinical institution is built to transform individuals into **Patients**. This maze is designed to deconstruct that passivity. You are not a vessel for data; you are the **Author** of your own somatic archive.

PATIENT



The Objective: Begin being a **PATIENT**. Navigate and become an **AUTHOR**. Stay away from the middle! Remember: the middle is a dead end of **average** outcomes and erasure. To survive, you must **aim for the tails**.

The Somatic Hazards: As you trace your path, look out for these icons. If you hit one, you have encountered a trap: # Bureaucracy, \$ Capitalism, * A Cold Winter, ~ The Waiting Room. The **AUTHOR** does not wait for the path to be cleared; they carve it out of the **poisonous knowledge**.

Ponle amor al té, en lugar de azúcar, toma y mira las estrellas.

Ponle amor al té, en lugar de azúcar, toma y mira las estrellas.

Day 1: The Infusion

This is my chemo cycle. It all starts on day one when they plug me into a machine and fill my body with life-saving poison; with liquid fire. I refuse the Latin etymology of the **Patient** - the one who suffers and waits. I am not waiting for the end of the infusion bag; I am taking authority over this chemical dialogue.

Liquid fire flows in.

Scorching soothes the unruly.

Come, ash-born flowers.

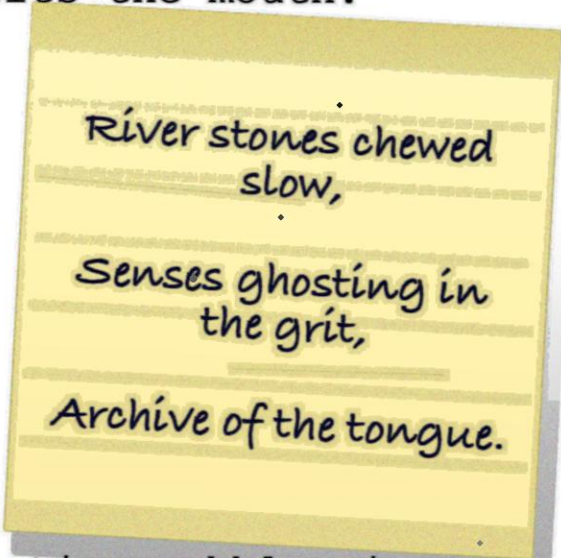
Even as the **FOLFOXIRI** cocktail (Oxaliplatin, Irinotecan, Bevacizumab, and Fluorouracil) enters the archive of my veins, I am acting. I have moved through 28 sessions of this somatic negotiation. One relentlessly every two weeks. Watching as a 13cm liver tumor dwindles to 9cm. This is the **poisonous knowledge** of the institution being repurposed. I aim for the 1% response that the doctors call **extraordinary**, because I refuse to exist in the middle of the curve. I **aim for the tails**.

The Somatic Constraint: Today, the environment must be room-temperature. The Oxaliplatin has rewritten the rules of my senses. Aggressive hydration is mandatory, but strictly avoid anything cold. To touch a chilled handle, a sip of cold water, a breath of cold air, is to trigger a pharyngolaryngeal spasm - the **silencing of the throat**. I am still the **Author** of this day, and I choose the warmth that keeps me vocal. I wrap up; leaving only my eyes uncovered. I challenge the cold outside to knock me down. It fails.

Con el vaivén del mar que va y viene, deja que te ame.

Day 2: The Pebbles

Day 2 is the day of peak sensory distortion. While the Dexamethasone's steroids are still active in the blood, providing a chemical shield against exhaustion, the Oxaliplatin and Irinotecan have reached their zenith in the nervous system. The **Difficult Knowledge** of this treatment is no longer just a concept; it is a physical texture that inhabits the mouth.



The sensation of eating pebbles is a somatic haunting; a rewriting of the ordinary act of nourishment. My tongue has become a troubled archive where the history of taste and texture is being overwritten by the chemicals lingering in my blood. To be a **Patient** is to simply endure the grit. As an **Author**, I choose to map this **poisonous knowledge** as a trace element of persistence.

FORM SG-09		SENSORY MAP	
Identify the textures you feel today. Check any poisonous knowledge currently inhabiting your somatic archive.			
☐ PEBBLES IN YOUR MOUTH	☐ METAL IN YOUR SKIN		
☐ ELECTRIC STATIC	☐ BODY OF GLASS		
☐ PHANTOM WEIGHT	☐ NUMBNESS		
☐ NOISE	☐ SILENCE		
Mantra: Aim for the tails. The middle of the curve encourages distortion as a side effect to endure; the Author documents it as a methodology.			

*Cúrate, mi niña, con amor del más bonito
y enciende el fuego.*

Day 3: The Persistence

Today I take my last pill. This is the final afternoon of the chemical shield. The last dose of Dexamethasone. While the buffer begins to exit the somatic archive, the **pebbles** remain: the gritty, persistent haunting of the tongue that refuses to be silenced. And I still need to wrap up and face the cold.

To be a **Patient** is to wait for the next dose, to endure the silence of the clinical interval. As an **Author**, I challenge the paradigm by actively attending my own clinical exit. I am not just a body being treated; I am the site of a chemical negotiation that insists on **persistence**. I listen to the silence of the shifting chemistry, **not as a victim** of side effects, but as a witness to the resilience of my own liver and body.

Clinical Archive: CT Record

Subject: 28th Cycle - Persistence Check.

Findings: Mixed response in liver [REDACTED].

Dominant metastasis at [REDACTED] currently 7.8 cm (reduction from 8.3 cm since Dec 2025). Segment 5 lesion shows slight [REDACTED].

Pulmonary status: Stable nodule.

Notes: Current measurements demonstrate authority over the initial Feb 2025 baseline (13.5 cm).

We do not wait for the **awesome window** to be given to us; we claim it through the grit of the Grind. **Mantra:** Aim for the tails. The middle of the curve expects a passive decline as the steroids leave. The **Author** uses the final glow of the buffer to prepare for the **transition**.

*Que se vuelvan polvo todos los dolores,
que los queme el fuego y vengan nuevas flores.*

Entrega tus dolores, que se vuelvan polvo
y vengan flores.

Activity 2: Letter Soup

Aim for the tails.
Searching for these words is not merely a distraction; it is an act of **formalized curiosity**. In the persistent noise of the clinical encounter, the **poisonous knowledge** of a Stage 4 diagnosis can feel like a chaotic wall of data that threatens to overwhelm you.

By identifying these specific words, you **challenge** that. You choose not to remain a silent, passive object of study. This activity is a practice of **re-membering** your own history.

A	U	T	H	O	R	I	T	Y	X	B	L	Q	P	R
S	O	M	A	V	Z	K	R	L	A	O	I	W	E	E
P	E	R	S	I	S	T	E	N	C	E	V	M	R	S
Q	L	J	G	B	O	R	D	E	T	S	A	K	S	I
B	O	R	D	E	R	S	F	Y	I	G	B	J	I	S
M	A	P	P	I	N	G	H	U	O	L	I	Z	S	T
C	D	E	F	G	H	I	K	J	N	M	L	P	T	A
L	I	V	A	B	I	L	I	T	Y	N	I	I	O	E
N	R	S	T	U	V	W	X	Y	Z	A	B	T	C	N
D	E	F	G	H	I	J	K	L	M	N	Y	O	C	E
A	R	C	H	I	V	E	P	Q	R	S	T	U	E	V
R	E	S	I	S	T	A	N	C	E	F	G	H	I	K
A	C	T	I	O	N	L	M	N	P	Q	R	U	W	X
T	A	I	L	S	Z	X	C	V	B	N	M	K	J	H
G	R	S	J	F	I	V	E	O	N	E	P	R	O	J

1. AUTHORITY 2. TAILS 3. SOMA 4. BORDERS 5. PERSISTENCE
6. ACTION 7. ARCHIVE 8. LIVABILITY 9. RESISTANCE.

stay away from the middle

Que se vuelvan polvo todos los dolores,
que los queme el fuego y vengan nuevas flores.

*Ponle amor al té, en lugar de azúcar,
toma y mira las estrellas.*

Day 4: Grounding Ritual

The Dexamethasone starts to recede. What follows is **The Detachment**: a spectral withdrawal where the self feels like a ghost haunting its own skin. Like the mind gets detached from the body. I do not wait for it to dissolve me; I perform a ritual of **grounding** to anchor the somatic archive back to the earth.

A RECIPE FOR REATTACHMENT

Prep time: 5 min

Cook time: 0 min

Serves: 1

Timing: Day 4, morning

INGREDIENTS

- 1 thick slice sourdough bread (non-toasted)
- 1 large egg
- 1 tbsp Avocado oil (for scrambling)
- 1 tbsp Extra Virgin Olive Oil (for drizzling)
- 1 slice Gouda cheese
- 1 tbsp sugar-free strawberry jam

STEPS

1. Heat Avocado oil over low-medium. Scramble the egg gently - do not overcook.
2. Place sourdough on a plate. Layer egg, then Gouda.
3. Drizzle generously with Olive Oil. Spread strawberry jam on one side.
4. Eat slowly. This is not just a meal; it is a grounding protocol.

Per serving: 480 kcal | Protein 22g | Fat 32g (mono. 18g) | Carbs 26g | Na 380mg

Purpose: Buffer carbs, stabilize blood sugar during steroid withdrawal, support GGT/ALP.

ANCHOR: If the detachment feeling is heavy, have a cup of Miso soup for its Magnesium and Sodium. And always aim for the tails.

*Cúrate, mijita, el dolor con el calor del sol
y el frío de la luna.*

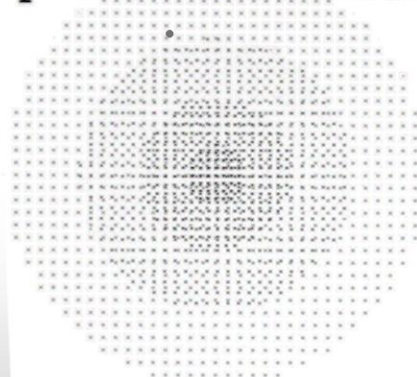
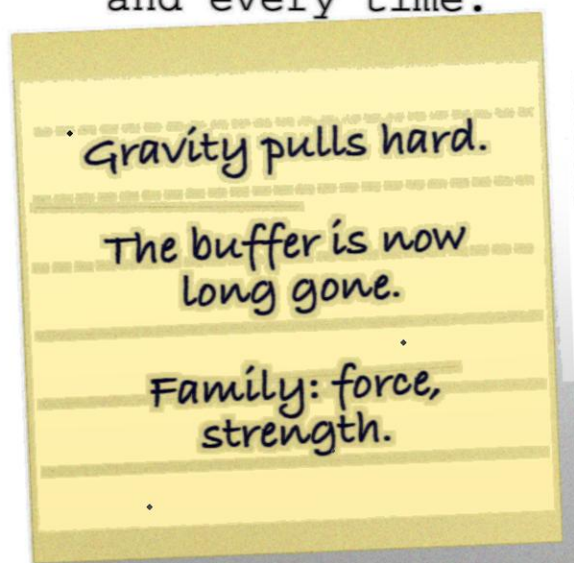
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Cúrate, mijita, el dolor con el calor del sol y el frío
de la luna.

Day 5: The Sunday Crash

Today, the buffer ends. The Dexamethasone has completed its exit from the somatic archive, and the steroid withdrawal hits with its full, gravitational weight. This is the **Sunday Crash**: the only day in the fourteen-day rhapsody where I feel impaired. The **difficult knowledge** of this journey is no longer a theoretical pursuit; it is the physical exhaustion that keeps me pinned to the bed.

Today I am the **somatic ghost**, but I do not haunt these parts alone. In the profound silence of the crash, that ever-present **family support** becomes the literal foundation of resilience. I linger because my home ensures the **ordinary** continues when I cannot. Because their living energy boosts my almost-depleted one. Each and every time.



Aim for the tails. Even in the quiet. **Especially** in the quiet.

"I am la medicina, but they are the hands that hold the cup."

Con el vaivén del mar que va y viene, deja que te ame.

*Endulza la mañana con aroma de lavanda
y que venga la calma.*

Day 6: The Monday Purge

Today is the **Monday Purge**; a day of peak systemic swelling. In the Latin etymology of the **Patient**, one would simply wait. **Endure**.

But we are not waiting; we are taking authority over our own circulation. The goal is fluid drainage and the management of retention through a strict 0mg added sodium diet and high potassium intake (bananas and coconut water).

This is another challenge to the paradigm within the self: refusing to let the body become a stagnant archive. Keep it **flowing**.

EXERCISE 1

ANKLE PUMPS

Lie flat on your back or sit with legs elevated above the heart.

Flex your feet up toward your shins as far as possible, then point your toes down as far as possible.

Reps: 20 slow
Freq: Every hour
Position: Legs elevated
Target: Lower extremities

EXERCISE 2

FIST-FANS

Extend your arms in front of you.

Clench your fists as tight as you can, then fan your fingers out as wide as they will go.

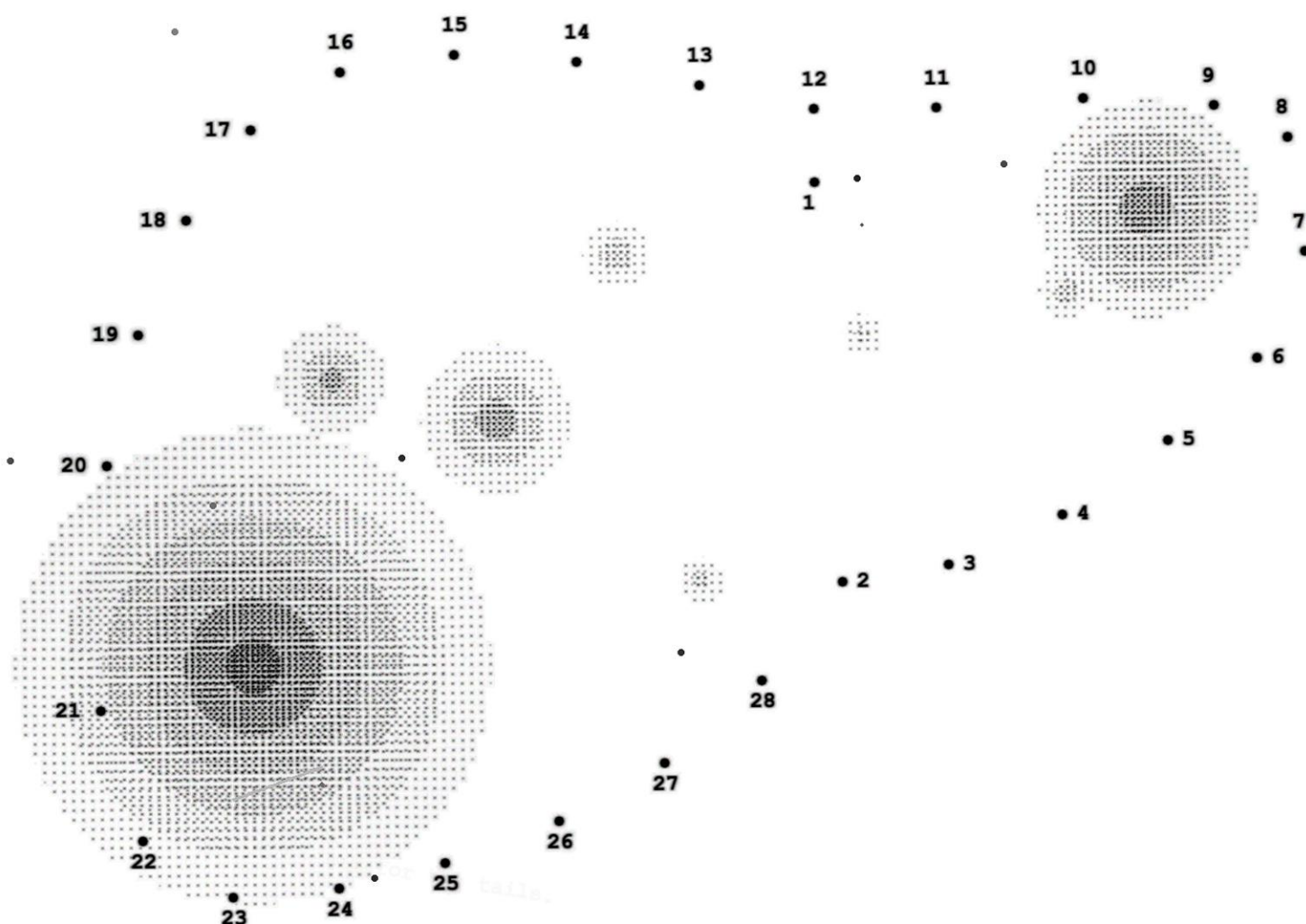
Reps: 20
Freq: Every hour
Position: Arms extended
Target: Upper extremities

Note: The Sodium Hard-Stop remains in effect.

Cúrate, mijita, con los besos que te sopla el viento.

Activity 3: The Somatic Map

The Task: Connect the dots below to reveal a shape. Follow the chronological timeline of your 28 chemotherapy cycles. As you draw the line, trace the history of your own **extraordinary results**. Each dot represents a day you stepped into the infusion room and chose action over patience.



The clinical archive records a reduction from 13cm to 7.8cm. Your counter-archive records the strength it took. Aim for the tails. The middle of the curve is a place of average outcomes; we are tracing the path of the 1% response.

*Mira el mundo con el ojo aquel
que lleva uste' en la frente.*

Cúrate, mi niña, con amor del más bonito,
y recuerda siempre que tú eres la medicina.

Day 7: First Light

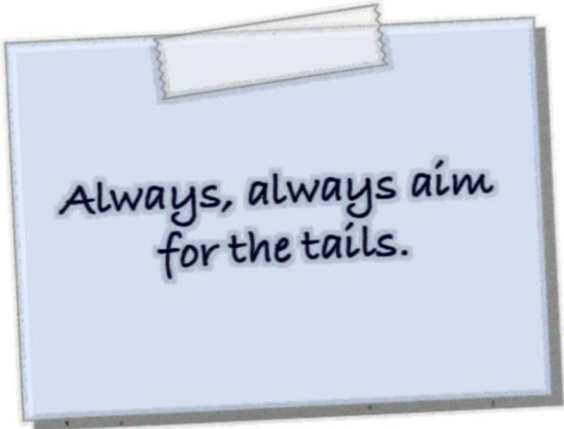
The **awesome window** begins to crack open today. Sometimes it waits until Day 8, but this cycle it arrives at 7. This is the moment of resurgence. The **reverberations of silence** that defined the crash are replaced by the first notes of unmedicated strength. I am starting to feel better without pills or buffers. The body is reclaiming its own rhythm. Sometimes it forgets it has cancer. That is a feeling to treasure.

As an **author**, I recognize this window as a primary site of resistance. This is where I re-occupy the **ordinary** and push back against the **atrocities** of the treatment. This is a **slow beginning** where the **poisonous knowledge** of the diagnosis is balanced by somatic knowledge. This clarity is a **shared achievement**.

Mira el mundo con el ojo aquel
que lleva uste' en la frente.

Days 8-14: The Awesome Window

The **grind** has receded, and the **awesome window** is now in full swing. But this window is not a gift; it is a space of resistance and reclamation. The ghost of the **patient** doesn't linger anymore at the corner of the eye. The **author** has fully taken over.



Authority is expressed through nourishment, awareness and care. I aim to support my liver's hard work and control my blood's **GGT** and **ALP** levels, keeping the bile duct pressure low. No more **poisonous knowledge** is allowed now (not until the next Day 1).

Navigating a terminal diagnosis as a trans-migrant body is not a solo performance. The **extraordinary results** of this 28-cycle journey are the product of a collective praxis of care. Their support makes the tails conquerable.

Grind
(rigid schedule)

Window
(flow, mix
and match)

1	2	3	4	5	6	7
+	⚡	◆	○	▼	∅	✦
8	9	10	11	12	13	14
🎵	🎵	🎵	🎵	🎵	🎵	🎵

+

 Infusion

⚡

 Neuropathy

◆

 Steroids

○

 Detachment

▼

 Crash

∅

 Purge

✦

 First Light

🎵

 Study

🎵

 Music

🎵

 Nourishment

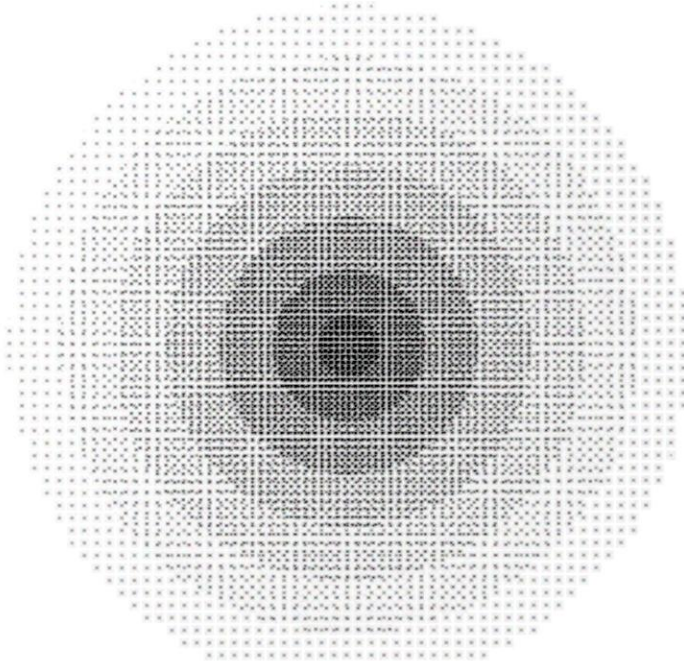
♥

 Family

Endulza la mañana con aroma de lavanda
y que venga la calma.

*Cúrate, mi niña, con los besos
que te sopla el viento.*

Coda



**"How do you tell a story that cannot be told,
yet must be told?" - M. NourbeSe Philip**

I am not a **patient**. I am an **author**. And I am the **archive**. This fourteen-day rhapsody is not a linear progression from illness to health; it is a rhythmic descent into the ordinary where the extraordinary becomes the pulse of daily life. To be an **author** is to recognize that **poisonous knowledge** does not have the final word. The final word is written in the **awesome window**, in the **grounding** of a shared meal, and in the **collective care** of the family and community that also refuse to let this somatic ghost haunt rampant. The **archive**, however, is never finished. Every 14 days, the record begins again. Every 14 days, we choose **action** over **patience**. Every 14 days we **aim for the tails**.

Final Activity: The Outlier Check

How many times did you see the phrase **aim for the tails** in this zine? Your count: _____

(Note: If you didn't count, you were looking at the middle of the curve. Go back. Find the outliers. They are where the persistence lives.)

*Que se vuelvan polvo todos los dolores, que los queme el
fuego y vengan nuevas flores.*

The Somatic Glossary

Aim for the Tails: Directive to seek agency in statistical outliers. A rejection of the normative center where bodies are managed and averaged.

Awesome Window: Days 8 through 14 of the cycle. A period of resurgence where the body is its own self even without medication, and the reverberations of silence fade.

Bevacizumab: Monoclonal antibody targeting vascular endothelial growth factor (VEGF). Administered on Day 1 as part of the infusion cocktail to inhibit tumor blood supply.

Counter-Archive: This record, constructed against and alongside the normative medical archive. This zine is a document of resistance authored by a body that refuses to be a patient.

Detachment, The: The sensation of withdrawal that begins on Day 4 as Dexamethasone exits the system. Starts as a mild thinning of reality and grows into a heavy, spectral disconnection from the body.

Dexamethasone: A high-potency steroid used as a buffer to manage the immediate side effects of the infusion. Its withdrawal on Day 4 triggers The Detachment, a state of somatic haunting where the self feels disconnected from the physical body.

Difficult Knowledge: A knowing that unsettlingly connects to violence or atrocity. In this archive, it is the 28 cycles of liquid fire that insist on being told.

Fluorouracil (5-FU): Antimetabolite chemotherapy agent administered via continuous infusion pump. Part of the FOLFOXIRI protocol that enters the "archive of the veins" on Day 1.

FOLFOXIRI: One of the most potent chemotherapy regimens. The combination protocol of Fluorouracil, Oxaliplatin, and Irinotecan. The three agents that enter the somatic archive on Day 1, each carrying its own form of poisonous knowledge.

Gaussian Distribution: The "normal" curve. In this zine, it represents the normative architecture of probability where most bodies are managed at the center. The Author refuses the mean.

GGT/ALP: Gamma-glutamyl transferase and Alkaline phosphatase. These are the somatic markers of bile duct

pressure. We monitor these trace elements to ensure the liver remains a functional archive of resistance.

Grind, The: The first 7 days of the chemotherapy cycle, characterized by infusion, steroid buffers, neuropathy peaks, and the "Sunday Crash."

Irinotecan: Topoisomerase inhibitor used in the FOLFOXIRI chemotherapy regimen. One of the three agents that enter the somatic archive on Day 1 of each cycle.

Ordinary, The: A conceptual framework by Veena Das describing the descent into the mundane after a catastrophic event. In this zine, it is the integration of life-saving poison into the daily rhythm of eggs, sourdough, and work.

Oxaliplatin: A platinum-based chemotherapy agent that induces acute peripheral neuropathy, characterized primarily by extreme sensitivity to cold. It creates a high risk of pharyngolaryngeal dysesthesia (spasm) if cold liquids or air are encountered, requiring strict adherence to room-temperature hydration.

Patient / Author: The central dialectic of this zine. From the Latin *patiens* (one who suffers / waits), the Patient is a state of normative passivity. The Author is the refusal of that passivity. The Author is the reclamation of agency over one's own somatic archive.

Pharyngolaryngeal Dysesthesia: A spasm of the throat triggered by cold exposure during Oxaliplatin treatment. It is the literal silencing of the voice by the machine, and the body's warning to stay in the warmth.

Poisonous Knowledge: A concept from Manduhai Buyandelger describing knowledge that acts as a destructive force, unsettling the individual's world. The diagnosis of Stage 4 cancer is poisonous not just biologically, but epistemologically.

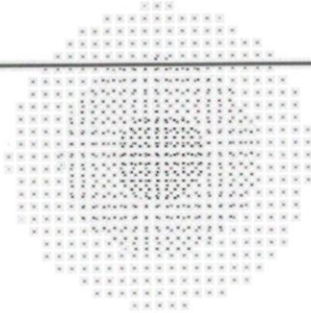
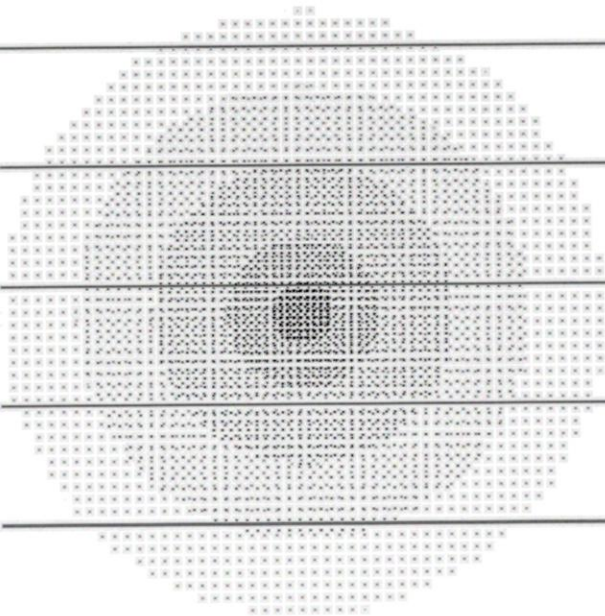
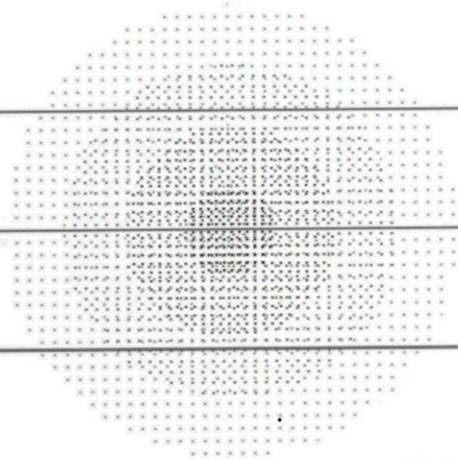
Somatic Ghost: The spectral state of the self during the steroid crash. On Day 5, this body becomes a ghost haunting its own skin. It is present but disconnected. The title of this zine reclaims that haunting.

Sunday Crash, The: Day 5 of the cycle. The only day of true impairment, when the steroid buffer has fully exited and the gravitational weight of withdrawal keeps the body pinned. Survival depends on the infrastructure of collective care.

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GRSJ 511 | Winter 2026

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